

TRANSACTION ANALYSIS

Walgreens Boots Alliance × Sycamore Partners

A Take-Private Driven by Strategic Pressure, Not Just Price

Announced March 6, 2025 · Closed August 28, 2025 · Healthcare M&A Analysis

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WBA

SYCAMORE PARTNERS

TAKE-PRIVATE · 2025

Four conclusions from this transaction

Walgreens / Sycamore is not a standard take-private. The structure, the timing, and the post-close design each reveal something important about healthcare strategy, retail pharmacy, and PE underwriting.

The Transaction Reflects Public-Market Exhaustion With a Complex Retail-Healthcare Turnaround

01

Structure tells the real story

The \$11.45 cash plus up to \$3.00 contingent DAP right reveals exactly where the two sides could and could not agree on value.

02

Multi-year decline made this feasible

WBA's equity value collapsed from earlier highs to a level that made a leveraged take-private financially viable for a PE sponsor.

03

Sycamore is buying control, not a clean story

Post-close: five separate standalone companies. The thesis is simplification and optionality, not a consolidated growth platform.

04

A signal for healthcare-adjacent retail

Scale did not protect value. Healthcare adjacency raised strategic ambition and execution complexity faster than it created valuation clarity.

This Is a Landmark Signal for Healthcare-Adjacent Retail and Private Equity Underwriting

01

RETAIL PHARMACY RESET

Scale no longer guarantees value

WBA's ~8,700-store US footprint did not insulate it from reimbursement compression, margin erosion, or equity collapse. Size is not a moat.

02

DEAL STRUCTURE AS SIGNAL

Contingent value reveals the negotiation line

The DAP right structure is a precise map of where buyers and sellers agreed, and where they could not agree. This design will be studied.

03

PE UNDERWRITING

Control creates optionality where public markets see only risk

Sycamore engaged where public equity had given up. The post-close five-company structure is the thesis: discrete assets, managed separately.

02

SECTION II

Deal Facts & Process

Transaction overview · Consideration structure · Timeline · Governance

Sycamore Agreed to Acquire Walgreens in a Transaction Worth Up to \$23.7 Billion

ACQUIRER	Sycamore Partners
TARGET	Walgreens Boots Alliance, Inc.
ANNOUNCEMENT	March 6, 2025
CLOSE	August 28, 2025
CASH CONSIDERATION	\$11.45 per share (at closing)
CONTINGENT VALUE	Up to \$3.00 per share, non-transferable DAP right tied to VillageMD-related asset monetization
TOTAL VALUE	Up to \$23.7 billion
FINANCING CONDITION	None, fully committed financing secured at signing
SHAREHOLDER VOTE	Required and obtained

Source: WBA press release March 6, 2025; WBA 8-K filed March 10, 2025; WBA completion announcement August 28, 2025

Note: \$23.7B includes equity consideration plus assumed net debt and other liabilities absorbed by Sycamore. Equity value at \$11.45 cash consideration is approximately \$9.9B (approximately 864M shares outstanding).

Shareholder Value Is Split Between Cash Certainty and Contingent VillageMD Monetization Upside



DAP = Divested Asset Proceed right. Non-transferable right to receive proceeds contingent on future monetization of VillageMD-related assets post-close.

The 29% Cash Premium Rises to Up to 63% When Contingent Consideration Is Included

CASH PREMIUM

29%

\$11.45 vs. \$8.85 reference

TOTAL PREMIUM (UP TO)

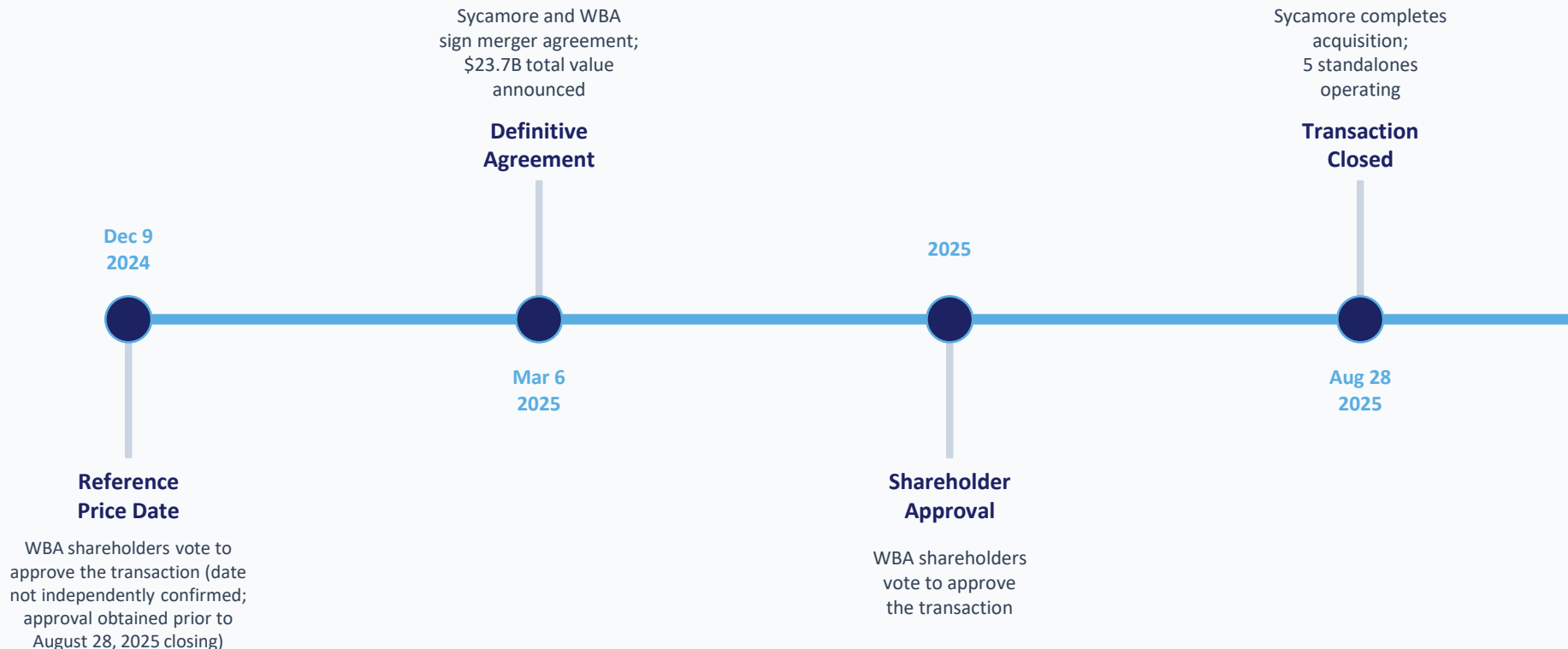
63%

\$14.45 vs. \$8.85 reference

PREMIUM REFERENCE TABLE

Metric	Value
Reference date	Dec 9, 2024
Reference price	\$8.85
Cash consideration	\$11.45
Cash premium	29%
DAP right (max)	\$3.00
Total consideration (max)	\$14.45
Total premium (max)	Up to 63%

The Process Moved From Market Rumor to Signed Merger to Completed Take-Private Within 2025



Committed Financing, No Financing Condition, and Shareholder Vote Reduced Execution Risk

CONFIRMED**No financing condition**

The definitive agreement contained no financing condition. Sycamore's financing was fully committed at the time of signing, removing a key execution risk for WBA shareholders.

CONFIRMED**Fully committed financing secured at signing**

Per the WBA press release, Sycamore had fully committed financing in place when the merger agreement was executed on March 6, 2025.

CONFIRMED**Shareholder approval required and obtained**

The transaction required WBA shareholder approval. Per the completion announcement of August 28, 2025, the acquisition was completed, confirming approval was obtained.

STANDARD**Regulatory clearances**

Transaction proceeded through standard regulatory review process. Closing August 28, 2025 confirms required clearances were received.

03

SECTION III

Why Walgreens Was Vulnerable

Equity reset · Operating headwinds · Store footprint · Capital actions

WBA Entered the Deal After a Multi-Year Public Equity Value Destruction — Down ~92% From Its Peak

2015 PEAK

~\$100

WBA near all-time high
~\$90B+ implied market cap

DEC 9, 2024 — REFERENCE DATE

\$8.85

~92% below 2015 peak
WBA-stated deal reference price

CASH CONSIDERATION

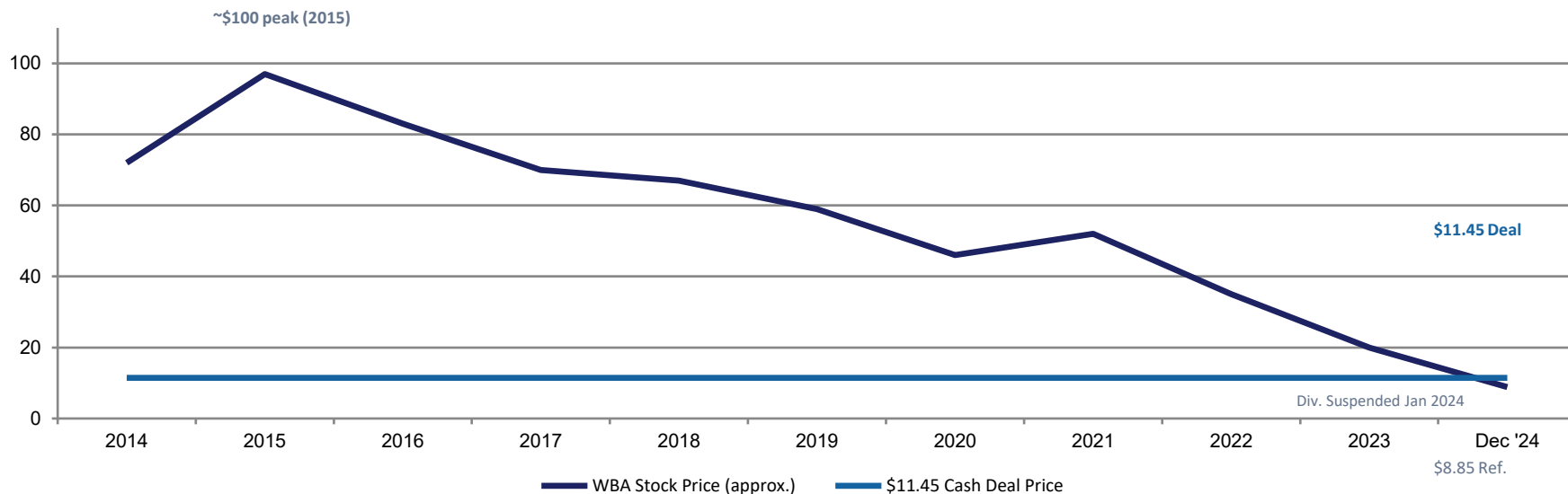
\$11.45

29% premium to reference
Announced March 6, 2025

WHAT DROVE THE COLLAPSE

- Pharmacy reimbursement compression: PBM DIR fees and script rate cuts progressively eroded pharmacy margin, the core earnings driver for retail pharmacy.
- Healthcare platform write-downs: VillageMD and Summit Health-CityMD goodwill impairments totaling billions, years of strategic capital with uncertain return.
- Dividend suspension (January 2024): Public signal that capital preservation had replaced growth investment; market responded with continued equity pressure.

WBA Equity Decline Was Structural, Not Cyclical — A Decade-Long Compression From ~\$100 to Single Digits



ANALYTICAL READ

- The decline was not one shock — it was sequential: reimbursement compression from PBM DIR fees, VillageMD/Summit write-downs erasing healthcare platform investment, dividend suspension in January 2024 signaling capital preservation mode, and loss of market confidence in any near-term standalone recovery.

Pharmacy Reimbursement Pressure and Retail Softness Left Little Room for a Public-Market Turnaround

01

Reimbursement Compression

PBM DIR fees and script reimbursement rate cuts progressively eroded pharmacy margin. Rate pressure was structural, not recoverable without fundamental model change.

02

Front-End Retail Decline

Consumer trade-down, foot traffic loss to mass retail and e-commerce, and promotional dependency weakened front-end contribution and reinforced the pharmacy margin squeeze.

03

Healthcare Platform Losses

VillageMD and Summit Health-CityMD generated significant operating losses and non-cash write-downs, absorbing capital while delivering no near-term earnings contribution.

04

Store Rationalization Burden

WBA announced closure of ~2,500+ US stores, generating lease obligations, restructuring charges, and execution risk rather than immediate earnings relief.

The Legacy Store Base Became a Restructuring Problem as Much as a Competitive Asset

US STORE LOCATIONS

~8,700

Largest US pharmacy chain by footprint

PLANNED US CLOSURES

2,500+

Phased closure plan announced 2024–2025

FIXED-COST DRAG

High

Closed stores carry lease commitments even post-closure

Why Size Became a Liability

Density of stores created a fixed-cost base that reimbursement rate compression could not support. Productivity per store declined as foot traffic migrated online and to mass retail. Size amplified margin pressure rather than offsetting it.

Closure Economics Are Complex

~2,500+ closures generate one-time charges, lease termination costs, and employee separation costs. Near-term P&L pressure materializes before any structural benefit, compressing reported earnings further during the transition.

Weak Earnings Quality and Restructuring Needs Eroded Confidence in an Extended Standalone Recovery

01

Reimbursement-Driven Margin Erosion

Pharmacy gross margin compressed as PBM contract rates tightened. DIR fee impact was structural, not recoverable without a fundamental change to the reimbursement model.

02

Healthcare Segment Operating Losses

VillageMD and Summit Health-CityMD generated negative operating income, absorbing WBA corporate resources while posting billions in goodwill impairments over FY2023–2024.

03

Restructuring Charges Obscuring Core Performance

Multi-year restructuring program created an ongoing gap between GAAP and adjusted metrics, making near-term recovery difficult to benchmark for public market investors.

04

Cash Flow Under Structural Pressure

Capital allocation to healthcare adjacency constrained free cash flow. Declining pharmacy EBITDA combined with restructuring spend left limited room for dividend maintenance or debt reduction.

Prior Capital Decisions Signaled That Walgreens Was Already Moving Into Preservation Mode

PERIOD	CAPITAL ACTION	SIGNAL
2021–2022	Multi-billion investment in VillageMD; \$3.5B WBA capital into Summit/CityMD acquisition	Healthcare expansion, growth thesis still intact
2023	Sequential dividend cuts	Capital preservation signal, payout no longer sustainable at prior levels
Jan 2024	Dividend suspended entirely	Explicit signal: standalone recovery not near-term; market responded with continued equity pressure
2024	Accelerated store closure program (~2,500+ US closures announced)	Shift from growth to asset rationalization mode
Mar 2025	Definitive agreement with Sycamore Partners at \$11.45/share	Resolution: private ownership chosen over continued public restructuring

THE LINE OF DEMARCATION

- The January 2024 dividend suspension was the public-market signal that standalone recovery was not near-term. The take-private that resulted in March 2025 began in the environment created by that signal.

04

SECTION IV

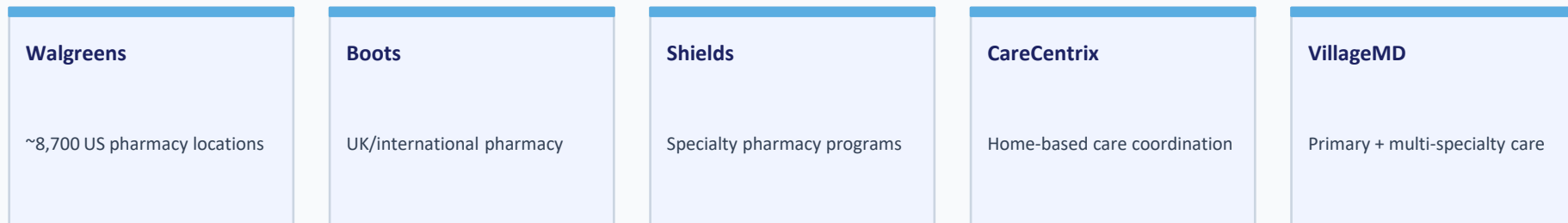
Healthcare Strategy & Asset Complexity

VillageMD · Summit Health-CityMD · DAP right logic · Portfolio structure

Walgreens Spent Years Building a Healthcare Platform Beyond Core Retail Pharmacy



POST-CLOSE PORTFOLIO — 5 STANDALONE ENTITIES



VillageMD Shifted From Strategic Growth Engine to Monetization Variable Inside the Deal Structure

VillageMD as Growth Engine (2019–2022)

- Strategic rationale: Integrate primary care at Walgreens locations, differentiated health destination model
- Investment thesis: WBA's ~8,700-store network creates unmatched patient access at scale
- Execution: Multi-year co-location buildout, controlling stake acquired 2021, then Summit expansion in 2022



VillageMD as Deal Variable (2025)

- Performance reality: Operating losses at scale; multi-billion goodwill impairments on Summit acquisition
- Strategic status: Not a near-term earnings contributor, a long-duration restructuring asset
- Deal role: Valued separately via DAP right (up to \$3.00/share), not included in base \$11.45 cash

WHY VILLAGEMD BECAME THE DAP RIGHT

- The bid-ask gap was precisely this asset. Sycamore couldn't price VillageMD monetization with confidence; WBA shareholders expected value from years of investment
- The DAP right (up to \$3.00/share contingent on VillageMD-related asset monetization) was the solution, deferring hard-to-price value rather than forcing a false precision argument.

The Summit Health-CityMD Deal Scaled the Platform But Increased Execution Burden and Value Uncertainty

01

What Summit Health-CityMD Was

Multi-specialty physician group + urgent care platform with ~700+ combined locations. CityMD: metro-area urgent care. Summit Health: multi-specialty physician practices. Acquired by VillageMD in November 2022 for ~\$8.9B valuation.

02

What It Added to WBA Strategy

Expanded from primary care into multi-specialty care and urgent care, broadening the patient journey addressable by the WBA healthcare platform. Created a potential end-to-end care model tied to pharmacy at scale.

03

What It Cost WBA

~\$3.5B Walgreens capital invested. Integration complexity across three distinct care models. Goodwill impairments as performance fell short of targets. Significant operating losses added to an already-pressured P&L.

TRANSACTION

VillageMD / Summit Health-CityMD

DATE

November 7, 2022

TOTAL VALUE

~\$8.9B

WBA CAPITAL

\$3.5B invested

NET EFFECT

- The Summit/CityMD deal expanded the platform's ambition while creating execution complexity that proved difficult to manage inside a publicly-traded, primarily-pharmacy company under margin pressure.

Sycamore Carved Out Uncertain Healthcare-Asset Value Rather Than Fully Pricing It Into Base Cash

BASE CERTAINTY — \$11.45 CASH

\$11.45

per share — paid at closing

Pays for: Core retail pharmacy, Boots, Shields, CareCentrix

Priced with: PE restructuring discount on operational distress

Certainty: High, paid at closing, no conditions

CONTINGENT UPSIDE — UP TO \$3.00 DAP

Up to \$3.00

per share — contingent on post-close outcomes

Pays for: VillageMD-related asset monetization proceeds

Priced with: N/A, deferred until monetization occurs

Certainty: Variable, depends on timing and form of realization

WHY THE SPLIT STRUCTURE WAS NECESSARY

- The DAP right is not a bonus, it is the mechanism that made the deal possible. Without it, either Sycamore overpays for uncertain assets, or WBA shareholders receive no credit for years of healthcare investment.
- Both parties accepted the value uncertainty and agreed to price it after the fact, replacing a false precision argument with a rational deferred-pricing solution.

The Structure Suggests Value Existed, But Not Enough Clarity for the Market to Pay for It Upfront



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SECTION V

What Sycamore Is Underwriting

Restructuring logic · Core pharmacy thesis · Post-close portfolio · Simplification

Sycamore Is Buying Time, Control, and Restructuring Flexibility

FACTOR	AS A PUBLIC COMPANY	AS A SYCAMORE PORTFOLIO CO.
Earnings cadence	Quarterly pressure; market reacts to near-term misses	Multi-year operating plan; no quarterly earnings penalty
Restructuring speed	Constrained by analyst optics and proxy risk	Accelerated execution, no headline risk on difficult decisions
Store rationalization	Closure announcements create negative press and employee friction	Portfolio optimization proceeds at optimal economic pace
Healthcare asset decisions	Each asset announcement amplified by public market reaction	Asset-by-asset value creation on Sycamore's timeline
Capital allocation	Public cost of capital; equity dilution optics constrain choices	PE capital structure optimized for restructuring, not optics
Exit optionality	Constrained to public market thesis and investor expectations	IPO, sale, spin, or hold, maximum flexibility per asset

Walgreens Remains a Large, Essential Asset Despite Its Depressed Valuation

Walgreens

Core US Pharmacy

~8,700 US locations; essential healthcare access; hundreds of millions of scripts annually

Boots

UK/International

Leading UK health & beauty retailer with pharmacy; meaningful international presence

Shields

Specialty Pharmacy

High-margin specialty pharmacy programs embedded in hospital systems, differentiated niche

CareCentrix

Home-Based Care

Post-acute care coordination and home health management, growing payor-contracted services

VillageMD

Primary Care Platform

Primary + multi-specialty care (incl. Summit/CityMD); DAP right upside; restructuring optionality

THE SCALE ARGUMENT

- At \$11.45/share, the lowest valuation in decades, Walgreens is still a \$23.7B+ transaction. The equity market discount created an entry point, not an absence of scale.
- The core pharmacy network services millions of patients and maintains PBM contract leverage despite reimbursement pressure. At normalized margins, the asset base supports a differentiated PE restructuring thesis.

Post-Close Standalone Operation of Five Entities Points to Portfolio Logic, Not Unified Platform

Walgreens

US PHARMACY

Value driver: Script volume; rationalized footprint

Sycamore Partners

Portfolio Owner

CareCentrix

HOME CARE

Value driver: Post-acute growth; payor contracts

Boots

UK/INTERNATIONAL

Value driver: Brand; European optionality

Shields

SPECIALTY PHARMACY

Value driver: Margin; hospital partnerships

VillageMD

PRIMARY CARE

Value driver: DAP right; platform optionality

Separation creates accountability. Each standalone entity has a distinct P&L, management team, and value story — rather than being blurred inside a complex multi-segment public company.

Sycamore Does Not Need Immediate Growth Heroics if Operational Cleanup Can Restore Optionality

5

CAPITAL EVENT

Exit on Improved Clarity

Public markets will price a simplified, profitable Walgreens differently than a distressed multi-segment healthcare platform

4

ASSET OPTIONALITY

Clarify Standalone Values

With each entity operating independently, discrete valuations become achievable, enabling partial sales, spin-offs, or IPOs

3

HEALTHCARE
RESOLUTION

Resolve Platform Losses

Exit or restructure VillageMD/Summit positions where DAP right economics are favorable; stop the operating loss bleed

2

PHARMACY
ECONOMICS

Improve Unit Economics

Optimize scripts per location, renegotiate PBM contracts, reduce DIR fee exposure, restore pharmacy EBITDA per store

1

FOUNDATION

Stabilize Core Pharmacy

Rationalize store base, renegotiate leases, reduce fixed-cost structure, stop the cash erosion before building anything new

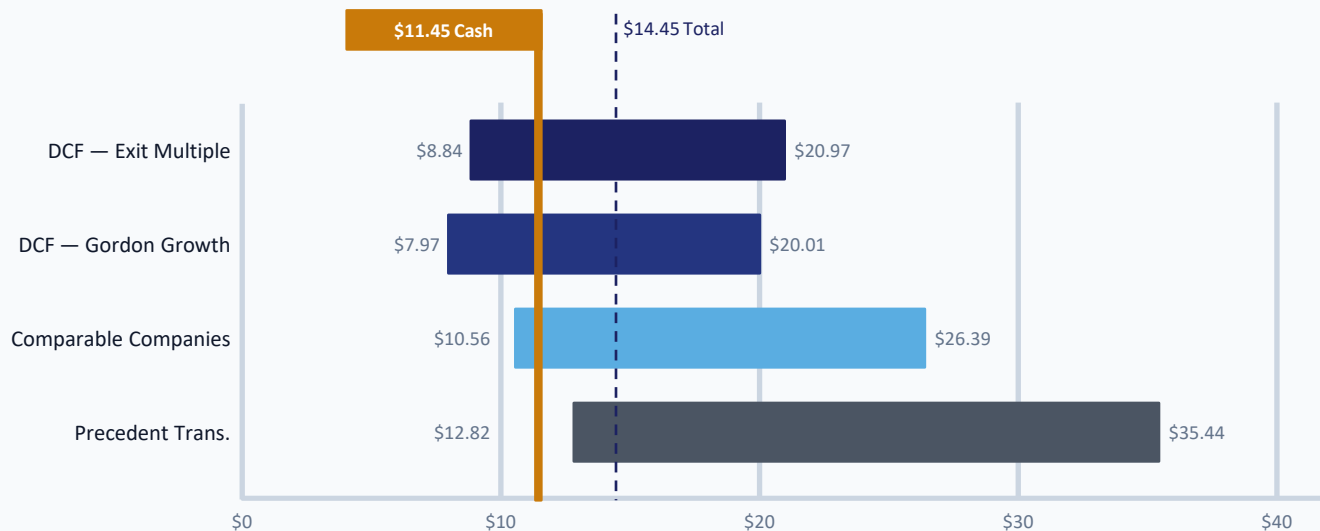
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SECTION VI

Valuation & Framing

DCF · Comparable companies · Precedent transactions · Deal structure

Three Independent Valuation Methods Show Deal Priced at or Below Intrinsic Value — Consistent With a Distressed Take-Private



- Deal EV/EBITDA: ~5.1x (cash) / ~5.8x (total), well below comparable precedent median of ~9x, reflecting operational distress and PE restructuring risk.
- DCF analysis: \$11.45 falls within the range at the lower end, consistent with PE buying at distressed WACC/multiple assumptions.
- The DAP right (up to \$3.00/share) separately captures VillageMD optionality, a mechanism no static model fully replicates.

WBA Was Acquired at ~5.1x EV/EBITDA — A Material Discount to Comparable Precedents; DAP right Bridges the Gap

Methodology	Implied Range (\$/sh)	Deal Cash (\$11.45)	Assessment	Key Driver
DCF — Exit Multiple	\$8.84 – \$20.97	\$11.45	Within Range	Base-case implies \$17.93; deal at lower third, PE distress discount
DCF — Gordon Growth	\$7.97 – \$20.01	\$11.45	Within Range	Terminal value sensitive to WACC–TGR spread; base case above deal
Comparable Companies	\$10.56 – \$26.39	\$11.45	Within Range	Peer set (CVS, distributors) structurally stronger; WBA distress discount applied
Precedent Transactions	\$12.82 – \$35.44	\$11.45	Below Range	Deal at 5.1x vs. precedent median ~9x, WBA distress + PE discount = alpha capture

ANALYTICAL CONCLUSION

- \$11.45 cash sits within DCF and comps ranges but below the precedent transactions range, the strongest evidence of an opportunistic multiple (~5.1x EV/EBITDA vs. ~9x precedent median).
- ~380bps EV/EBITDA gap vs. clean precedent median reflects WBA-specific distress: reimbursement compression, healthcare platform losses, and the PE restructuring discount Sycamore required.
- The DAP right (up to \$3.00/sh) is the structuring innovation: Sycamore paid for pharmacy certainty while retaining healthcare platform upside, bridging the bid-ask gap.

WBA Was Acquired at a Material Discount to All Comparable Companies — Operational Distress and PE Restructuring Risk Drove the Gap

COMPANY	CATEGORY	EV/EBITDA (APPROX.)	BASIS	RELEVANCE TO WBA
CVS Health (CVS)	Pharmacy Retail + PBM + Health Insurance	~8x	Public market consensus, FY2024	Closest direct comparable — pharmacy retail + healthcare services; CVS also under margin pressure in 2024
McKesson (MCK)	Pharmaceutical Distribution	~14x	Public market consensus, FY2024	Distribution overlap; premium reflects higher-margin, asset-light model vs. WBA retail mix
Cencora (COR)	Pharmaceutical Distribution	~14x	Public market consensus, FY2024	Distribution; structurally cleaner earnings and more predictable margin than WBA
Cardinal Health (CAH)	Distribution + Specialty Pharma	~12x	Public market consensus, FY2024	Distribution with specialty mix; lower multiple vs. McKesson/Cencora reflects segment complexity
Rite Aid (RAD)	Pharmacy Retail	N/A — Ch. 11 Oct 2023	Filed for bankruptcy; public market data	Closest operational comparable by store model; illustrates downside of scale without reimbursement pricing power
WBA — This Deal	Pharmacy Retail + Healthcare Adjacencies	~5.1x	Deal EV/EBITDA — cash consideration (\$11.45)	Subject company; ~380bps below comps median — PE distress + restructuring discount

COMP SET READ

- The ~380bps gap between WBA's deal multiple (~5.1x) and the comp median (~9x, see slide 29) reflects not just operational distress but the PE restructuring premium Sycamore demanded for full control and long hold flexibility. Distributors trade at a premium to retail pharmacy on earnings quality; the Rite Aid Chapter 11 marks the floor scenario WBA's public equity was beginning to price.

A Business Once Valued Far Higher Became Acquirable Because the Market Lost Faith in the Turnaround Path

2015 PEAK VALUATION

~\$90B+

WBA market cap near peak, ~\$100/share stock price at all-time high

DEC 9, 2024 EQUITY VALUE

~\$7–8B

Reference date market cap
92%+ compression from 2015 peak

DEAL TOTAL VALUE (MAX)

\$23.7B

Up to \$14.45/share total
(incl. DAP right upside)

WHY THIS COMPRESSION CREATED THE PE OPPORTUNITY

- The equity market didn't 'miss' the value, it priced the turnaround risk correctly given the public-company constraints it operates under.
- No public investor could offer Sycamore's combination: full control, long time horizon, no quarterly earnings obligation, and the ability to restructure without public market penalty.
- The gap between the equity market's price and the deal value is the PE control premium, not a market error. It's the cost of conditions under which a turnaround is actually possible.

The Split Consideration Design Reveals the Negotiation Boundary Between Immediate Value and Hard-to-Price Assets

EQUITY MARKET FLOOR

\$8.85

Dec 9, 2024 reference close
(WBA-stated reference date)

Where public equity priced WBA given
all visible headwinds, the negotiation floor

CASH CERTAINTY

\$11.45

Per share at closing
29% premium to reference

Core pharmacy + international assets
priced with PE restructuring discount

CONTINGENT UPSIDE (DAP)

Up to \$3.00

DAP right, non-transferable
VillageMD-related monetization

The gap both parties acknowledged
but could not price at closing

THE NEGOTIATION LOGIC

- The DAP right is not a sweetener, it is the structural solution to an honest disagreement about asset value.
- Both parties accepted that VillageMD-related assets had value, but not '\$X value' at closing. The contingent mechanism replaced a false precision argument with a deferred-pricing solution that made the deal executable.

07

SECTION VII

Implications for Healthcare M&A

Platform lessons · PE appetite · Valuation dislocation · Asset optionality

Scale Alone Does Not Protect Value When Reimbursement and Consumer Economics Deteriorate

ASSUMPTION	WHAT WBA SHOWED	IMPLICATION
Scale creates pharmacy pricing moats	~8,700 stores did not protect pharmacy margins, PBMs control reimbursement rates regardless of network size	Size is a cost structure, not a pricing advantage, if PBMs control the margin pool
Pharmacy drives defensible, recurring cash flow	Script reimbursement is structurally declining; PBM power grows with consolidation	Recurring revenue does not mean growing revenue; pharmacy is a service, not a product moat
Retail pharmacy is a natural platform for adjacency	Healthcare adjacency raised complexity and consumed capital faster than it created value	Platforms require operational mastery before strategic extension, the sequence matters critically
Public markets will fund long-duration healthcare turnarounds	WBA had to go private to escape quarterly earnings pressure during the restructuring	Public market patience windows are shorter than healthcare transformation timelines

Integration Across Care Settings Raised Complexity Faster Than It Created Valuation Clarity

HIGH COMPLEXITY + HIGH VALUE CLARITY

The promised land, where integration premium exceeds complexity discount.
NOT where WBA ended up.

HIGH COMPLEXITY + LOW VALUE CLARITY

WHERE WBA ENDED UP.
Multiple segments, multiple care models, each valued differently,
none valued cleanly inside a distressed public company.

LOW COMPLEXITY + HIGH VALUE CLARITY

Where Sycamore wants each standalone entity to be.
Simple story, clear EBITDA, defensible multiple.

LOW COMPLEXITY + LOW VALUE CLARITY

Where distressed retail pharmacy sits.
Revenue is observable, earnings are not.

← LOW COMPLEXITY

HIGH COMPLEXITY →

The DAP right and the 5-standalone post-close structure are Sycamore's solution to the valuation opacity created by complexity. Separation makes each asset independently priceable.

This Deal Shows PE Will Engage Where Public Markets See Too Much Uncertainty, if Control Can Create Options

WHAT PE REQUIRES	WHAT WBA OFFERED	WHAT PUBLIC MARKETS COULD NOT PROVIDE
Full control → restructuring flexibility	100% ownership post-closing; no minority shareholders	No control buyer alternative in public market
Long time horizon without quarterly EPS pressure	4–7 year private ownership; no disclosure calendar	Quarterly pressure with a declining near-term trajectory
Asset-level optionality on PE timeline	5 standalone entities with distinct value paths	Market valued them bundled; no clean standalone story
Absorb restructuring pain without IR constraints	Full P&L control; decisions made for economics, not optics	Each restructuring charge amplified by public market reaction
Clear thesis at asset level (not platform level)	DAP right separates uncertain healthcare value from core pharmacy	Could not price healthcare platform independently from distressed core

PE didn't see something public markets missed. They offered something public markets structurally cannot: the conditions under which a turnaround is actually possible.

The Post-Close Structure Suggests Discrete Assets May Matter More Than the Old Combined Narrative

Walgreens

VALUE DRIVER

Rationalized US pharmacy footprint; script volume after closures; PBM renegotiation optionality at reduced cost base

Boots

VALUE DRIVER

International brand with strategic optionality; European health & beauty market position; natural buyer universe among global retailers

Shields

VALUE DRIVER

Specialty pharmacy margin profile; deeply embedded in hospital systems; acquirable by a major health system or PBM at a significant premium

CareCentrix

VALUE DRIVER

Post-acute care coordination growth; payor-contracted services with durable revenue; beneficiary of home-based care shift in Medicare/Medicaid

VillageMD

VALUE DRIVER

DAP right upside contingent on monetization; restructured primary care platform; Summit/CityMD optionality as platform or as asset sales

M&A Lesson: When a company cannot be valued coherently as a whole, separation becomes the value creation strategy. The post-close standalone structure IS the plan.

Each asset valued independently > combined valuation as a distressed public company

08

SECTION VIII

Closing Synthesis

Investment view · Structural lessons · Follow-up questions

Walgreens Went Private Because Restructuring Complexity Outgrew What Public Markets Were Willing to Fund and Wait For

THE CORE THESIS

Walgreens did not simply choose to go private. Its operating pressure (reimbursement compression, healthcare losses, store rationalization), strategic complexity (five business units each requiring separate resolution), and healthcare portfolio ambiguity (VillageMD value not priceable in public markets) made private ownership the more credible restructuring path.

WHY SYCAMORE?

Sycamore offered what no public-market outcome could provide: full control, PE-grade restructuring capital, a multi-year time horizon with no quarterly obligation, and the ability to separate five standalone entities on a value-maximizing schedule rather than a disclosure calendar.

WHAT THE DEAL SIGNALS

Three things: (1) Scale in retail pharmacy is no longer a durable moat, reimbursement compression is structural, not cyclical. (2) Healthcare adjacency at the platform level introduces valuation complexity that public markets price as a discount, not a premium. (3) When value exists but clarity does not, PE will engage, and the structure of how they engage reveals the negotiation boundary precisely.

The Next Value Questions Sit Around Monetization, Separation, and Operating Repair

Q1

VillageMD Monetization Path

How and when will Sycamore monetize VillageMD-related assets? IPO, sale to a health system, or private hold? This drives the DAP right payout for former WBA shareholders.

Q2

Boots Strategic Review

Is Boots a long-hold asset or early sale candidate? A major global pharma or health & beauty retailer could be a natural buyer at a meaningful premium to embedded carrying value.

Q3

US Store Rationalization Progress

How many of the ~2,500+ planned US closures have been executed? What is the post-closure unit economics for remaining locations, and when does the EBITDA benefit materialize?

Q4

Shields & CareCentrix Standalone Value

Are Shields and CareCentrix on a path to IPO or trade sale? Both operate in niches with active buyer universes, specialty pharmacy and post-acute care respectively.

Q5

Core Pharmacy Margin Recovery

Can WBA renegotiate PBM contracts more favorably post-rationalization? What is the realistic cadence of pharmacy EBITDA margin recovery over a 3–5 year private ownership window?

Q6

Exit Horizon and LP Return Profile

Given PE fund lifecycle, which assets are likely sold first to return LP capital? Does the multi-asset structure allow for sequential monetizations over an extended hold period?

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SECTION IX

Appendix

Source log · Transaction timeline · Business profile · Asset reference · Methodology

Source List & Methodology

PRIMARY SOURCES

WBA Press Release, Definitive Agreement

Mar 6, 2025

Transaction terms, consideration structure, premium percentages

WBA 8-K Filing

Mar 10, 2025

Transaction mechanics, DAP right terms, financing structure

WBA Completion Announcement

Aug 28, 2025

Close confirmation; 5 standalone entities confirmed

SUPPLEMENTAL

WBA Investor Materials (DAP Right)

Mar 2025

Contingent consideration structure detail

WBA 10-K Annual Report FY2024

FY2024

Operating metrics, segment results, store count, restructuring

WBA Investor Materials

Various

Store count (~8,700 US); healthcare strategy; closure plans

REFERENCE

VillageMD / Summit Health-CityMD Announcement

Nov 7, 2022

\$8.9B valuation; \$3.5B WBA capital invested

Public Market Data

Historical

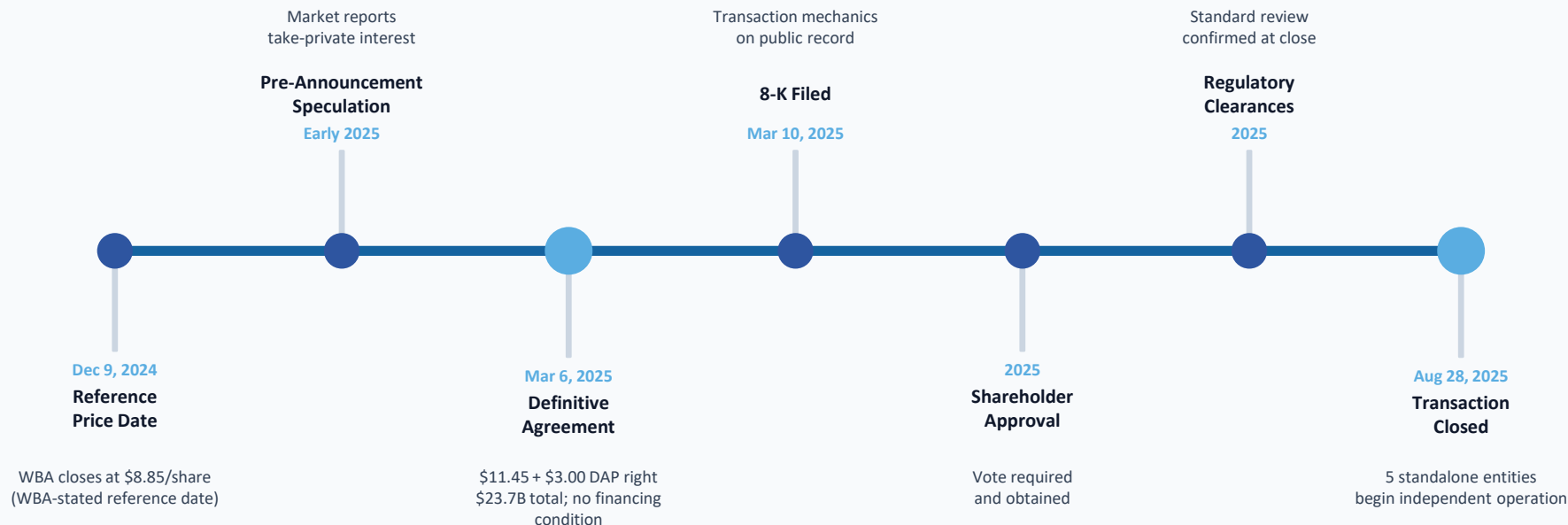
Approximate historical stock prices for context only

WBA SEC Filings

Various

Financial exhibits and valuation model cross-reference

Detailed Transaction Timeline



Walgreens Boots Alliance — Business Profile Snapshot

~8,700

US Walgreens Locations

Per WBA investor materials

~300K+

Global Employees

Approximate per WBA materials

5

Post-Close Standalone Entities

Confirmed Aug 28, 2025

COMPANY BACKGROUND

Incorporated:	Walgreens Boots Alliance, Inc., formed via 2014 merger of Walgreens (est. 1901) and Alliance Boots
International:	Boots (UK/Intl), leading UK health & beauty pharmacy chain
Key Services:	Retail pharmacy, prescription dispensing, specialty pharmacy, primary care, home care coordination
Post-Close:	Private, Sycamore Partners acquired August 28, 2025; five standalones operating independently

POST-CLOSE SEGMENT OVERVIEW

	Walgreens	~8,700 US pharmacy locations; script volume + front-end retail
	Boots	UK/International health & beauty pharmacy chain
	Shields + CareCentrix	Specialty pharmacy + home-based care coordination
	VillageMD	Primary + multi-specialty care; Summit/CityMD; DAP right asset

VillageMD / Summit Health / CityMD — Asset Reference

VillageMD

- Founded: 2013
- Model: Primary care clinics co-located at Walgreens pharmacies
- WBA relationship: Initial partnership 2019; WBA became controlling shareholder 2021
- Role in deal: Monetization variable; addressed via DAP right (up to \$3.00/share)
- Post-close status: Standalone entity under Sycamore portfolio

Summit Health

- Model: Multi-specialty physician group practices
- Scale: ~700+ Summit/CityMD combined locations at acquisition
- Acquired by VillageMD: November 2022 (same transaction as CityMD)
- Total acquisition value: ~\$8.9B
- Strategic role: Scaled VillageMD into multi-specialty, added complexity

CityMD

- Model: Urgent care centers across major metro areas
- Acquired alongside Summit Health: November 2022
- Focus: Convenient, walk-in urgent care, complementary to primary care
- Integration challenge: Distinct care model from VillageMD primary care
- Post-close: Part of VillageMD standalone entity

Transaction Terms & Mechanics — Quick Reference

DEAL TERMS

TRANSACTION TYPE

Take-private merger, WBA acquired by Sycamore Partners

ANNOUNCEMENT DATE

March 6, 2025

CLOSING DATE

August 28, 2025

CASH CONSIDERATION

\$11.45 per share, paid at closing

CONTINGENT VALUE

Up to \$3.00/share, non-transferable DAP right

MAX TOTAL PER SHARE

Up to \$14.45 per share

TOTAL DEAL VALUE

Up to \$23.7 billion

PREMIUMS & STRUCTURE

REFERENCE PRICE

\$8.85/share, December 9, 2024 (WBA-stated)

CASH PREMIUM

29% to reference price (\$8.85 → \$11.45)

TOTAL PREMIUM (MAX)

Up to 63% including \$3.00 DAP right

FINANCING CONDITION

None, fully committed financing at signing

SHAREHOLDER APPROVAL

Required and obtained

POST-CLOSE STRUCTURE

Five standalones: Walgreens · Boots · Shields · CareCentrix · VillageMD

Stock Price Reference Points — The Decline and the Deal

